

Cardiovascular

Angina			MI	Pericarditis	Cardiac Tamponade
-Chest pain, opressive substernal, crushin radiated to the left side Relive with Nitro			- The Same Do not Relieve - Confirmatory Diagnostic TROPONIN -Atypical Manifestation * Woman -Fatigue, Epigastric Pain *Elderly -LOC/Alteracion/ Confusion *Diabetes -Painless Treat: M- Morphina O-O2 N-Nitro A-ASA -Thrombolytic Therapy (6H) *Alteplase *Tenecteplase  Bleeding Brain Hemorrhagic Stroke CT-Scan • Fue Efectivo - Porque aparece un PVC After MI= Revascularization Nota: UAP camina a un pt con MI 24h After	- The Same Pain relieve in leaning forward Avoid: -Worseing in supine position -worsening with inspiration and swallowing (Aumenta con cambios de posicion) EKG -ST Segment elevation -Pericardial Friction Rub Treat: -NSAID -Colchicine -Antibiotic in case of infection -Pericardiectomy (ultimo)	- Muffled heart sound -JVD with CLEAR LUNG - Paradoxal pulse de 130/90 a 110/90 Baja 10 - Pericardiocentesis
Chronic	Unstable	Prinzmetal			
-Dolor en el pecho con ejercicio Treat: -Nitro	-Sentado dolor en el pecho Treat: -Nitro	-Dolor que te levanta en la madrugada Treat: -Diltiazem -Verapamil -Nifedipino EKG: -ST Segment Elevation Negative T			

ISCHEMIC CARDIAC DISEASE

CHF (congestive heart failure)

Right Side	Left Side = LUNG
-Edema -JVD - 45 Grados - Semi Fowler -Ascites -Hepatomegaly/Splenomegaly -↑Urinary output	-SOB -Lung Crackle (base) -Tachypnea -Hemoptisis - Pink Frothy Sputum (Emergency) - Ascending Crackles more than half of both lungs -Orthopnea -Paroxystic nocturnal Dyspnea

Diagnostic CHF

- BNP>100

Treatment

- Sentarlo (Position)
- Diuretic (Furosemide)
- Nesiritide (UNCOMPENSATED CHF)
- Iminrinone (UNCOMPENSATED CHF) (↓BP)
- Digoxin (Toxico +2)
(Halo vision and N/V/D)
- Lol and Pril to cardiac protective

Cardiogenic Shock	Endocarditis (infeccion, inflamation)
-Hypotension + Tachycardia -Diaphoresis, coldness, pallor -Urinary output -LOC / ALT / Confusion -Sign of pulmonary congestion Treat: -Dopamine -Dobutamine -IV fluid -swan-ganz catheter -Distal port to check PCWP 6-12 mmHg >12 -Proximal port CVP 3-7 >7	Causa: -IV Drug abuses -Associated with dental procedure 1-2 week before dental procedure -Valvular protesís - Rheumatic fever Antibiotico profilactico Assessment: -Fever -Heart murmur - osler's node -janeway lesion -splinter hemorrhage Complication: -Septic embolism CVP measurement -Central line RA Magnetometer or Transducer 0- o point - at the phlebostatic Axis 4th Intercostal (RA)

	Complication: *Septic Embolism - Stroke - Pulmonary Embolism -Renal Embolism - Splenic embolism Treatment: - Antibiotico- PIC line (4-6 weeks)
--	---

Nota: Central Venus Catheter - Medicamentos

Nota: COR Pulmonary = Complication COPD
 Symptoms = Right Side CHF

Valve Replacement

Mechanic	Biological
• Metal (more risk for thrombosis) -Life Long Anticoagulation -Lasting 30 years • Plastic	Graft • pig or cow (xenograft) • Human cadaver (homograft)

Cardiac Sound (Aprietime)

A
 (aortic)
 2nd ICS right

P
 (Pulmonary)
 2nd ICS left

T **M**
 5to ICS left 5to ICS left
 Close to the Sternum border Mild Clavicular line

Vascular Disorder

Arterial	Venous
1- Arterial Insufficiency 2- Raynaud 3- Buerger's Disease Thromboangiitis obliterans	1- Phlebitis 2-Thrombophlebitis 3-Venous Insufficiency 4-DVT 5-Varicose vein (nunca a salido)

1- (legs dangling)	1- Cause IV
--------------------	-------------

-Intermittent claudication (walking until the point of pain - rest and the advance a litter further -Hairless -Thickened toe nail -↓ or absent peripheral pulse - Colden, cyanosis -Ulcer - Between toes or in upper aspect to feet - Dry - Base pale, no granulation tissue - Pain full - Risk for necrosis, amputation= worse prognosis - Rest pain - Buttock discomfort 2- (Bandera Americana) Associated: - Cold Exposure -Stress -Caffeine intake - Smoking ● Vasoconstriccion (Blue or white) ● Vasodilation (Red) Board: Ask about occupation? Treatment: 1. Washer (warm water o use gloves) 2. Stop smoking and stop caffeine 3. Calcium channel blockers -Diltiazem - Verapamil - Nifedipino 3- SMOKING -Intermittent claudication -Ischemic digits pain while at rest (more severe at night) -Ulcer same to arterial insufficiency -Necrosis and frequent Amputacion Treat: STOP SMOKING	Assess = hot, rednem (No subir brazo) Treat= Stop IV, warm moist compress 2- Cord formation or streak through a vein wall. Same to Phlebitis (no subir) 3- Assessment Tiene edema , Skin pigmentación or stasis dermatitis or brown skin pigmentation (PIE CARMELITA) Ulcer: -Ankle -Moist -Uneven Edge - Base pink/ granulation tissue - Better prognosis Treat: 1. SCD - UAP (medias de compresión) 2. Avoid prolonged sitting or standing 3. Use sequential compression device (twice daily x 1 hour) 4. Cause -Immobility (long travel, obesity, surgery, pregnant, oral contraceptive, smoking) Assessment 1.Unilateral calf pain 2. Positive homan's signs Treat: -Prevention Early Ambulation -If Present Bed rest (72h) Elevated extremity -Thrombolytic Therapy (5-7 days after) -Warfarin prophylactic
---	---

NOTA: (AWAKENS AT NIGHT)

- Epigastric pain = Duodenal ulcer
- Butock pain = Arterial Insuficiency

- Rigors pain = Buerger's Disease
- Big toe pain = Acute Gout
- Chest pain = Prinzmetal Angina

Aortic Aneurysm

- Thoracic - es de arriba- sign respiratorio
 - SOB - Cyanosis - Tachycardia - Hoarseness (Priority)
 - Abdominal
 - Pulsate / Abdominal mass
 - Bruits over Epigastrium
- Complication
- Rupture
 - Lower back pain
 - Hypotension + Tachycardia

DO NOT PALPATE

Associated with Marfan

ANEMIA

Iron Deficiency	Megaloblastic	Aplastica	SCA	Talasemia
1. Intermittent bleeding 2. Chronic gastritis 3. Toddler (milk) 32oz 4. Pregnant (physiologic) Assessment 1. PICA 2. Palor, fatigue, Activity intolerance, SOB 3. Cheilitis / Cheilosis 4. Spoon Nails Diagnostic -Ferritin ↓ -Serum Iron ↓ -Transferrin ↑ -Hb, HCT ↓ -Reticulocytes (Sign early Improvement 15 days) Treat: 1. ↑ Iron in diet -Red meat	- Folic Acid Sulfa, Phenobarbital - Pernicious Vit B12 deficiency Causa -Elderly (Chronic gastritis) -Surgery -Vegan Signs -Red beefy tongue -Neurologic Symptoms (ataxia, balance disturbance, Paresthesia) Diagnostic -Schilling Test 24 h collection Treat: -Vit B12 - IM monthly life long	Causa -Chemotherapy -Radiation therapy Assessment ↓ WBC (Risk for infection) Treat: neutropenia or febrile ↓ Platelets (risk for bleeding) Treat: platelet transfusion ↓ RBC (risk anemia) Treat: Epoetin & Diagnostic -Bone marrow biopsy	-African American Assessment 1. Anemia 2. Vaso Occlusive crisis (pain full) 3. Priapism 4. Hand and foot syndrome (early) Triggers - Dehydration - Infection - High Altitude -Surgery Diagnostic -Hb-Electrophoresis Treat: H- Hydration O- Oxygen P- Pain medication (Morphine)	-Asia Mediterranean anemia Assessment -Greenish yellow skin tone -Severe anemia (blood transfusion frecuente) Complication: -Hemosiderosis or Iron overload Treat: -Chelation therapy -Dexferoxamine -Hemodialysis

-Organ meat -Shellfish -Gentils + Vit C to fix (orange,tomato juice) 2. Sulfate/Fumerant ferroso S/E Constipation 3. Iron Dextran (Z tract technique) 4. Blood Transfusion Hb less than 7				
--	--	--	--	--

Nota: Splenectomy: Vacuna - Pneumococcal and Influenza

Hemophilia	Von-Willebrand (autosomal Dominant 50%)	Idiopathic (thrombocytopenic purpura)
- Deficiency coagulation factor Hemophilia A VIII -Cryoprecipitate Hemophilia B IX - Fresh Frozen Plasma (Para los dos) Bleeding -More important joint bleeding (hemotrosis) -Ice pack and Immobilization Priority (Stroke hemorrhagic) Treat: -Bleeding precaution -No contact sport -Desmopressin	-Epistaxis -Gum bleeding -Excessive menstrual bleeding Treat: -Bleeding precaution -Desmopressin	Autoimmune (Unknown) -↓Platelets -Petechia, purpura, Equimosis - Weaken, fatigue, hypotension Treat: 1. Prednisone= Immunocompromised 2. IV = Immunoglobulin 3. Danazol 4. Splenectomy